



Community Mental Health Home Visiting SOP

Scope

Site	Department, division or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health Division (MHD), Community Mental Health Services (CMHS), including Statewide services and Wungen Kartup	Multidisciplinary (MDT)

Key alerts

Alerts must be added to Psychiatric Services Online Information (PSOLIS) / WebPSOLIS as required

Purpose

This standard operational procedure (SOP) aims to provide guidance to staff to ensure supportive, risk management strategies are in place that uphold the safety of all staff attending a consumer's home or a designated community setting.

Management of the risk to home visiting staff caused by potential exposure to infectious microorganisms, including respiratory viruses, gastrointestinal viruses and blood-borne viruses, should be in accord with the *RPBG Home Visitation Infection Prevention and Management (IPM) Clinical Guideline*.

Links to relevant documents

- [Take 5: Community Personal Duress Alarms \(IOTTAG GPS\)](#)
- [Home Visitation Infection Prevention and Management \(IPM\) Clinical Guideline](#)

Definitions

Community Visit	A period of time during which a staff member is required to carry out work in a consumer's home or a community setting outside the physical location of the CMHS. This applies to single or recurrent visits.
IOTAGG Duress Device (DD)	Personal duress alarms which have a built in GPS tracking and a duress button which are linked to a 24/7 staffed-call centre that will deploy emergency services to the GPS location when the duress alarm is activated by staff.

Procedure details

For guidance and policy directives to promote staff safety, clinicians working within the consumer's home/community setting are to:

- Apply this SOP in conjunction with the **East Metropolitan Health Service (EMHS) Community/Home Visiting Policy** and conduct a pre-home visiting risk assessment – Appendix I: Guidelines for Community/Home Visiting
- Complete annual mandatory Emergency Procedures and Aggression Prevention and Intervention (API) training
- Apply existing safety precautions and systems to support safety and complete orientation sessions / Take 5 in the use of the IOTAGG DD
 - Must wear IOTAGG DD during all community visits
- During a visit, staff are to respect and take into account the consumer's and/or their family/personal support persons (PSP) cultural and social diversity, including considerations pertaining to Aboriginal people, ethnicity, pregnancy, disability, religion, spirituality, belief, sexual or bodily diversity, gender identity, age, medical condition or other, specific cultural/personal considerations
- Provide information about the CMHS' mutual expectations and rules of engagement to the consumer, ideally during the admission process (e.g. welcome packs). Ensure the information is delivered in a way understandable to the consumer and seek consumer's agreement to the terms
 - Information about rules of engagement should include identification of appropriate behaviours (e.g., respectful boundaries/language, non-smoking, no alcohol/drug/substance use, no sexual misconduct/harassment/ aggression/ violence etc.) during the visit(s). Consumers should also be aware of the possible outcomes of inappropriate behaviours, such as discharge or police involvement
 - Periodically revisit the rules of engagement when breaches occur, including:
 - Threats of violence or aggression from the consumer to the clinician
 - Any activity by the consumer necessitating the activation of the IOTAGG DD
 - Any other risk issues that arise
 - Document the outcome of their communication of the CMHS mutual expectations and rules, and the consumers' amenability to these boundaries, as appropriate. The client's management plan should reflect the consumer's response/ compliance, and appropriate interventions, e.g., 2-person visits required if a consumer chooses not to agree to the terms, psychiatrist review, etc.
- Ensure students or volunteers are accompanied by RPBG staff
- Complete orientation of new/inexperienced staff relating to community home visiting and preferably, ensure their attendance on visits with experienced/senior staff.

Processes prior to leaving the CMHS

Clinicians are to:

- Between October and April (during the bushfire season) and on hot, dry and windy days, refer to the [EMHS Safety Alert for Bushfire Safety](#) relating to safe visiting process
- Check the IOTAGG DD status upon collection and ensure it is working properly
- Raise safety or risk concerns to relevant Team Leader/Clinical Lead/Consultant/Program Manager/On-call Consultant Psychiatrist After Hours and collaboratively develop a plan for the visit or make a decision if the visit is to proceed, as appropriate, and document in the consumer's integrated notes. This includes a decision as to whether **two staff members** are to undertake the visit. This is mandatory for:
 - **High-risk visits**
 - **All after hours (A/H) visits** (after 16:30hrs, including weekends or public holidays)
- Ensure the vehicle utilised has more than ¼ of a tank of fuel before leaving the CMHS
- Apply processes for respiratory virus (RV) infections including COVID-19 safety. Refer to **CMHS Provision: Responding to Respiratory Virus Illness SOP**.

Processes prior to leaving the CMHS cont'd

Community visit notification system

Clinicians are to complete the relevant community visit notification system that is in place for the specific CMHS' clinical setting for planned home visits, including:

For business hour home visits

- Notify Administration staff/enter the appointment on an electronic journey diary or white board and record visits in the chronological order that they are due to be completed, including:
 - Consumer's name, address, and telephone number
 - Destination (address)
 - Time of leaving
 - Expected return time (ETR) or completion of each visit
 - Apply IIOTAG DD escalation/notification, as required.

For A/H home visits

- **City East, Midland CMHS Assessment and Treatment Team (ATT) and Wungen Kartup** staff must notify Mental Health Emergency Response Line (MHERL) of home visit, including consumer details, address, ETR, vehicle and mobile contact details. If not returned by ETR time, MHERL to contact on mobile; if nil response then welfare check to WAPOL
 - Contact MHERL staff (Mon to Fri 1630–2200hrs and weekends/public holidays 0830–2200hrs) via email: MHERL@health.wa.gov.au/ Tel: **1300 555 788**
 - Ensure consumer's Crisis Safety Plan is up to date to facilitate response by MHERL, particularly for high risk consumers

Note: MHERL has limited scope of actions due to delivery of clinical service. CMHS staff are to apply IIOTAG DD escalation/notification process, as required

- **Bentley CMHS** has access to other A/H staff located on site. Staff are to notify BHS ATT of home visit with above details via switchboard or mobiles as per [ATT Hub page](#) until 2200hrs. After 2200hrs, notify the BHS A/H Mental Health (MH) Clinical Nurse Specialist (CNS) via switchboard or on Page 552.

Community MH home visiting procedure and escalation for clinician's safety

En-route / on arrival at the consumer's home

If the consumer or other persons at the home don't refrain from smoking, options may include:

- Meeting outside of the home
- Terminating the visit
- Inviting the consumer to attend a CMHS

Inside the location / consumer's home

In circumstances where a consumer may display inappropriate behaviour, e.g., intoxication (alcohol/drug/substance use), aggressive/violent, sexualised behaviour or other, inappropriate, harmful behaviour towards staff and/or others:

- Advise the consumer to immediately cease the inappropriate behaviour and if they do not, terminate the visit and invite the consumer to attend a CMHS at another time
- Activate an IOTAGG DD distress call if required (see Take 5: IOTAGG DD)
- At the earliest opportunity, provide the consumer with a copy of the CMHS' rules/mutual expectation information, communicate these expectations of behaviour/conduct in a way that is understandable to the consumer and seek their agreement

Progress of the visit

If staff become aware they will be delayed, notify

- During business hours, the designated contact in each clinical setting, e.g. On-Call Clinician/Team Leader/Triage or Duty Officer apply IOTAG DD escalation/notification
- During AH – CMHS staff are to apply IOTAG DD escalation/notification process (as required)
 - BHS CMHS staff - until 2200hrs, notify Bentley ATT via switchboard or contact directly on mobile as per [ATT Hub page](#). After 2200hrs, notify the AH CNS via switchboard/on page 552
 - City East, Wungen Kartup and Midland CMHS staff - notify MHERL Tel: 1300 555 788
- Provide a new estimated time of return and at least **every 30 minutes for high-risk consumers** as to the progress of the visit. Further follow-up should only occur after the stipulated time
- Record updated information in electronic journey diary/whiteboard/with Administration staff (e.g., changes to expected time of return to the CMHS and the reasons)

Note: If MHERL/AH CNS does not receive updates or notification of a clinician's return, they will attempt to contact the clinician and/or enact the local emergency response outlined above in locating the clinician.

If a clinician is more than 30 minutes overdue returning from a community visit

The designated contact in each clinical setting, e.g., Duty Clinician/Team Leader/Triage or Duty Officer / AH CNS /relevant CMHS ATT becomes the identified response coordinator and takes responsibility for safety monitoring/urgent emergency response for the relevant clinician. Staff are to take actions, including:

- Review the electronic journey diary/whiteboard /information via Administration staff to ascertain possible location of the clinician
- Contact the clinician on their work mobile number.
- **If successful** and clinician's safety is verified/confirmed, update clinician's arrival time on the CMHS' whiteboard/journey diary/Administrative staff
- **If clinician indicates they're unsafe**, inform Team Leader and refer to below actions/initiate emergency/distress call
- **If unsuccessful**, leave a voicemail message instructing the clinician to call back but continue to escalate to next step and continue attempts to contact the absent clinician
- Complete a search of the CMHS premises/car park, if relevant
- Contact Facilities Management to check if the vehicle has been returned, if appropriate
- Review the absent clinician's diaries (e.g., Outlook, hard copy, etc.) for related information regarding whereabouts
- Phone the absent clinician's personal/work mobile and continue to make contact
- Phone the consumer's contact number
- If no response is received,
 - During business hours, promptly inform Line Manager (i.e., Team Leader/Program Manager apply IOTAG DD escalation/notification (as required)
 - During A/H – City East, Wungen Kartup and Midland CMHS staff notify MHERL and apply IOTAG DD escalation/notification process (as required). MHERL to contact on mobile if nil response then welfare check to WAPOL. Bentley CMHS are to notify MH CNS (A/H) /BHS ATT
- Provide the clinician's last known location, the time of the last call, car registration and description, relevant history of the consumer/other relevant details

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Version: 4

Endorsed by: Mental Health Division Co-Director 24/01/2025

Authorised by: Clinical Practice Committee 06/02/2025

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Community MH home visiting procedure and escalation for clinician's safety cont'd

If a clinician is more than 30 minutes overdue returning from a community visit - continued

- The designated contact staff member is to initiate a visit with another clinician and/or additional staff support, if needed to the location listed for contact (with/without police)
- If the clinician is located ensure their safety. If clinician is not located and/or the incident has not resolved, initiate **emergency/distress call response** i.e. press and hold the SOS button on IOTAGG DD to activate an alert/call to emergency services or the CMHS that emergency assistance may be required
 - In circumstances where staff are unable to use/access IOTAGG, contact emergency services (e.g., police, ambulance) via mobile, informing them of the concerns and request their attendance at the clinician's location, as appropriate
- If a clinician has been missing for greater than 30 minutes and all risk mitigation strategies have been followed, the Program Manager/Clinical Lead/CNS AH is to inform Mental Health Division (MHD) Head of Department (HOD) and/or Medical Co-Director/delegate/ site executive on call
- Checks are to continue until contact is established or police have contacted the designated contact in each clinical setting with the outcome of their welfare check
- If the clinician returns to the CMHS the clinician is to inform the designated contact in each clinical setting/other staff of their return and indicate safety
- Notify the consumer's NoK/PSP if the consumer's safety is at risk
- Debriefing will occur and will be coordinated by the Program Manager, as appropriate

If a clinician escalates risks with visit/ makes an emergency help call to the CMHS

The designated contact in each clinical setting, e.g., Duty Clinician/Team Leader/Triage or Duty Officer / AH MH CNS /relevant CMHS ATT is to

- Advise the clinician to make direct contact with emergency services/distress response, if possible (e.g., IOTAGG DD, phone police via 000)
- Obtain the clinician's name, location and, if possible, the nature of the emergency
- Initiate emergency/distress response as outlined above/make direct contact with police/press the SOS button on IOTAGG SDD to activate an alert
- Contact the Team Leader or, if unavailable, Clinical Lead/Program Manager who are to inform the MH Division Medical Co-Director/delegate about the incident at the earliest convenience
- Communicate with CNSs of local hospitals and inpatient units, as appropriate
- Inform AH CNS/MHERL/ATT mobile and provide details as per the electronic journey diary/Administration staff /white board
- Inform consumers' NoK/PSP if the consumer's safety is at risk (via Care Coordinator/ Team Leader / Program Manager/ treating doctor)
- Checks are to continue until the clinician's safety is established or police have contacted the clinician/designated contact person/MHERL with the outcome of their welfare check
- Debriefing will occur and be coordinated by the Program Manager, as appropriate

If staff have not returned or phoned the CMHS by 1600hrs and the designated contact in each clinical setting, e.g. On-Call Clinician/Team Leader/Triage or Duty Officer / AH CNS /relevant CMHS ATT is unable to contact the absent clinician via their mobile

The designated contact staff member is to

- Immediately notify the Team Leader/Program Manager/Clinical Lead regarding the absent clinician
- Inform the MHD Medical Co-Director/delegate/site executive on call about the incident at the earliest convenience
- Inform the AH CNS via switchboard/page 552 or ATT via mobiles as per [ATT Hub page](#) (1630 to 2200hrs 7 days) for Bentley CMHS or for City East, Wungen Kartup and Midland CMHS, MHERL Tel: 1300 555 788/ apply IOTAG DD escalation/notification process and provide relevant details
- Inform the consumers' NOK/PSP if the consumer's safety is at risk
- Debriefing will occur and be coordinated by the Program Manager, as appropriate
- If the clinician returns/makes contact they are to inform the designated contact in each clinical setting/other staff of their return and indicate safety

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Welfare checks / urgent mental health assessment

Clinicians on site may determine the need for a welfare check/community mental health status assessment based on safety concerns regarding the mental state of a consumer in the community, and/or if they are unable to make contact with the consumer, they are to take actions as soon as practicable.

Escalate care to the Team Leader/Consultant, if required. Refer to **Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP**, **WA Department of Health Community Mental Health Status Assessments: Role of the Mental Health Clinicians Policy** and **EMHS Notifications Requirements under the WA Mental Health Act 2014 Policy**.

Note: Forced entry is a decision and the responsibility of police.

Returning to the CMHS

- Ensure the vehicle has more than ¼ of a tank of fuel before returning to the CMHS
- Activate a PSOLIS alert for identified potential hazards at the location, print and place it in the front of the consumer's integrated notes
- If a critical incident has occurred, access post-incident debriefing and support, as appropriate. Refer to the **EMHS Post Incident Management/Critical Incident Stress Debriefing Policy** and/or for consumer's post-critical incident debriefing, the **RPBG Mental Health Consumer/Carer/Visitor Support Post-Critical Incident SOP**.

Documentation

Complete documentation related to the community visit in the consumer's integrated notes, including:

- Actions identified/completed
- Any concerns/issues that may have prompted change in management prior, during or post the visit
- Interaction and outcomes of the visit (pre, during and post) and any, other related information/communication, updated risk assessment, alerts, client management plans/crisis awareness plans
- Escalation of decisions when decisions not to provide service are based on a risk assessment that has been discussed with the Team Leader/Program Manager
- Completion of a Datix Clinical Incident Management System (CIMS) report for any situation where the consumer was at risk of or received injury, and if there was a risk or incident to the clinician, complete a Combined Hazard or Incident Reporting (CHoIR) form and Notifiable Incident Reporting to the Office of the Chief Psychiatrist (OCP) report, as appropriate.

Compliance monitoring

Compliance with this SOP will be monitored as part of the:

- Datix CIMS system process and CHoIR and/or Notifiable Incident Reporting to the OCP, if appropriate
- RPBG Performance Monitoring Schedule reported by MHD Quality and Safety meetings
- Compliments/Complaints Management System.

Facilitator

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Aboriginal health impact statement and declaration

The voice of the Aboriginal community is reflected in the Community Mental Health Home Visiting Safety SOP through consultation with the Aboriginal Health Strategy Unit and Aboriginal Workforce Engagement Group to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 1586).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

Related policies, practice standards, clinical guidelines

[RPBG Policy Hub](#)

- [Transmission-Based Precautions: Use of Personal Protective Equipment \(PPE\) SOP](#)
- [Community Mental Health Services Triage to Discharge SOP](#)
- [Sexual Safety Policy](#)
- [Recognising and Responding to Acute Deterioration Policy](#)
- [Recognising and Responding to Family and Domestic Violence and Elder Abuse SOP](#)
- [CMHS Provision: Responding to Respiratory Virus Illness SOP](#)
- [Recognising and Responding to Acute Deterioration in Mental State, Cognition and Behaviour SOP](#)
- [RPBG Mental Health Consumer/Carer/Visitor Support Post-Critical Incident SOP](#)

Related policies, practice standards, clinical guidelines cont'd

EMHS Policy Hub

- [Care Coordination in Mental Health Services Policy](#)
- [Rights of Mental Health Patients Policy](#)
- [Duress Alarm Systems Policy](#)
- [Occupational Safety and Health Policy](#)
- [Injury Management Policy](#)
- [Prevention and Management of Workplace Aggression and Violence Policy](#)
- [Acute Response Mental Health Emergencies / Crisis Policy](#)
- [Notifications Requirements Under the WA Mental Health Act 2014 Policy](#)
- [Emergency and Disaster Management Policy](#)
- [Community/Home Visiting Policy](#)
- [EMHS Safety Alert for Bushfire Safety](#)
- [Notifications in Mental Health Policy](#)
- [Post Incident Management/Critical Incident Stress Debriefing Policy](#)

Department of Health Policy Hub

- [Safety Planning for Mental Health Consumers Policy](#)
- [Safety Planning Procedures for Mental Health Consumers](#)
- [Principles for the care of people who may be at risk of violent or aggressive behaviour](#)
- [Principles and best practice for the clinical care of people who may be suicidal](#)
- [Statewide Standardised Clinical Documentation \(SSCD\) Policy](#)
- [Clinical Incident Management \(CIM\) Policy](#)
- [Department of Health Community Mental Health Status Assessments: Role of the Mental Health Clinicians Policy](#)

Other

- [Chief Psychiatrist's Standards for Clinical Care](#)
- [Mental Health Act \(MHA\) 2014](#)
- [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [Safety Alerts - All Documents \(health.wa.gov.au\)](#) - Bushfire Safety Alerts
- [Work Health and Safety Act 2020](#)
- [Tobacco Products Control Act 2006](#)

Related national standards

ACSQHC NSQHS Standards 2nd Edition (2021):

Standard 1: Clinical Governance;

Standard 2: Partnering with Consumers;

Standard 5: Comprehensive Care;

Standard 6: Communicating for Safety;

Standard 8: Recognising and Responding to Acute Deterioration

NSMHS (2010):

Standard 2: Safety;

Standard 6: Consumers; clinical deterioration

Standard 10: Delivery of care